- Deliver close to EDD
- Frequent Blood Pressure, 24 hours urine, liver enzymes
- Fetal Heart Rate
- Ultrasounds.

Severe preeclampsia: BP = 160/110 mmHg, epigastric pain, 2-4+ proteinuria, ^ liver enzymes, thrombocytopenia [↓ 100,000].

Goal: prevent convulsions & control BP. Magnesium sulphate is the drug of choice

Magnesium Toxicity based on clinical signs: such as sharp drop in BP, respiratory paralysis and disappearance of patellar reflex.

- STOP infusion
- O_2 administration
- Calcium gluconate if magnesium sulphate toxicity present

Gestational Diabetes Mellitus

Glucose intolerance beginning in pregnancy.ussually present in the second or during the third trimester. Fasting blood sugar exceeds 90 mg/dl and post prandial value is greater than 120 mg/dl

Maternal risk	Infant risk
Hypertention disorders	Birth trauma
Pre term labour	Shoulder dystocia
Polyhydramnios	Hypoglycemia
Macrosomia (C/S rate)	Hyperbilirubinemia
	Respiratory distress syndrome
	Thrombocytopenia
	Hypocalcemia
	Fetal death

Pathophysiology

- Pregnancy hormones estrogen, HPL, prolactin, cortisol, progesterone, blocks insulin receptors > 20 weeks pregnancy.
- Results in increased circulating glucose
- More insulin released to attempt to maintain glucose homeostasis
- Patient feels "hungry" due to increased insulin
- vicious cycle of increased appetite & weight gain results

Diagnosis

- Oral Glucose Challenge Test (OGCT)
- Screen all women at 24-28 weeks.
- HIGHER Risk patient to be screened in 1st trimester/1st prenatal visit and at 24-28 weeks.

Determining High Risk Clients

- Family history DM; Previous GDM
- Marked obesity; Glycosuria
- Maternal Age > 30
- Previous infant > 4000g
- Member of high-risk racial/ethnic group
- Hispanic, Native American, South or East Asian, African American, Pacific Islander.
- If results negative, repeat during 24-28wks.

Interventions

Antepartum Goal: strict glucose control.

- Provide immediate education to patient. and family members
- Standard diabetic diet [2000-2500 cal/day].
- Total calories – 30Kcal/Kg for normal weight women

Distribution of calories: 40-50% carbs, 20% protein, 30-40% fat,

Recommend: 3 meals & 3 snacks evenly spaced to avoid swings in blood glucose. Snack at bedtime.1200 mg/day calcium, 30 mg/day iron, 400 mcg/day folate.

Intrapartum: monitor glucose levels and titrate with insulin

Postpartum: Mostly return to normal after delivery.

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